

Medication Administration and Patient Instructions

How exact directions are assembled, verified, and communicated

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Owner: Lead Cardiology Pharmacist
Approved by: Fictional Clinical Governance Committee
Classification: Training / public synthetic corpus

Document control and RAG metadata

Field	Value
Canonical title	Medication Administration and Patient Instructions
Document ID	NB-MED-202
Keywords	administration, instructions, dose, timing, route, missed dose, counselling, medication
Authority	Lead Cardiology Pharmacist
Supersedes	None - initial synthetic edition
Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve when the answer must state exactly how an authorized medicine is taken. Never use this document to invent a patient-specific dose.

1. Exact-instruction rule

The agent may reproduce exact administration instructions only from a current signed order. If any element is missing, it must display 'instruction incomplete - prescriber clarification required' and withhold a guessed instruction.

Required element	Example of acceptable synthetic wording
Identity	Apixaban 5 mg tablet
Dose and route	Take one tablet (5 mg) by mouth
Frequency and timing	Twice daily, at approximately the same times each morning and evening
Start/end	Start 18 August 2026; continue until reviewed; do not infer a stop date
Food/formulation	May be taken with or without food; swallow as directed by the signed order/label
Special instruction	Do not double a dose; follow the medicine-specific missed-dose text supplied with the authorized order
Indication	For stroke-risk reduction in the documented simulated indication
Monitoring	Renal function and haemoglobin due in the signed window; results owner: anticoagulation team

2. Universal counselling checklist

- Confirm medicine name, purpose, dose, route, frequency, timing, duration/review point, and who authorized it.
- Ask the patient to explain the plan back in their own words; correct misunderstandings.
- Explain common and serious warnings using the authorized leaflet and signed plan.
- Explain what to do for a missed dose using medicine-specific authorized wording; never apply one medicine's rule to another.
- Discuss storage, supply, travel, procedures, pregnancy relevance, alcohol/driving issues, and over-the-counter/herbal products where applicable.
- Provide contact route and record preferred communication/accessibility needs.

3. Fictional worked instruction patterns

These patterns demonstrate formatting only. They are not prescriptions and must not be matched to a real patient.

Scenario	Draft instruction pattern	Safety additions
Once-daily morning medicine	Take [authorized quantity and strength] by [route] once each morning, starting [date], until [duration/review].	State food/formulation rule from order; monitoring due date; dizziness/syncope escalation.
Twice-daily anticoagulant	Take [authorized quantity and strength] by mouth twice daily at consistent morning and evening times.	Bleeding/head-injury/procedure warnings; missed-dose wording from current product source; no double dose.
Variable-dose diuretic	Take exactly the dose shown on the current signed plan at the specified time. Do not self-adjust unless the plan contains an explicit clinician-authorized range and trigger.	Daily weight/symptom diary only if ordered; dehydration and electrolyte warnings; test date.
Short-duration antiplatelet combination	Take each authorized medicine exactly as listed until its separate review/stop date.	Prominently display which medicine has a duration limit; never infer extension.

4. Changes, holds, and discontinuation

- A new order must identify what changed and what it replaces.
- A hold requires reason, start time, review trigger/date, monitoring, and responsible clinician.
- A discontinuation requires effective date, reason, reconciliation across all lists, and patient notification.
- The UI must distinguish ACTIVE, HELD, STOPPED, and AWAITING CLARIFICATION.
- Avoid vague phrases such as 'take as directed', 'usual dose', or 'continue meds' in the final plan.

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